NCCP 00279 · clinician draft

Cisplatin + etoposide with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin Day 1 · high emetic risk

Active medicines: Cisplatin + Etoposide

Emetogenic risk: High

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1;80 mg orally Days 2–3	Before SACT then daily Reference course; check interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk and the whole multiday prescription.
Dexamethasone	12 mg orally Day 1;8 mg daily Days 2–4	Reference high-risk NCIS course Reconcile all steroids; no duplicate orders.
Olanzapine	5 mg orally daily Days 1–4	Reference high-risk NCIS course Review sedation and additive extrapyramidal risk with metoclopramide.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days The high-risk row does not specify an end day/PRN qualifier; prescriber specifies duration and suitability.

Additional phase-specific support

- Pre/post cisplatin hydration under local policy. Etoposide1000mL may supply prehydration; oral Day1 etoposide cannot.
- Source suggested posthydration:10mmol magnesium sulphate and20mmol potassium chloride in1000mL saline over2hours; electrolytes/volume must be checked against patient status and local infusion policy.
- Routine furosemide is not advised unless fluid overload; mannitol only according to local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00279 v8 · pNCCPpp1–2,5;NCISp4 · NCCP NCIS medical-oncology antiemetics v8

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required